How accurate are analyst forecasts for 1st entry rare disease products?

- **Analyze set of rare disease 1st to market products (launched within last ~10 years)**
  - Track analyst peak revenue estimate ranges prior to launch
  - Compare to actual sales and current consensus forecasts
  - Prior to launch, are analysts "under- or over-calling" $ opportunity for 1st to market rare disease products?
  - Explore explanations for misses; focus on products where learnings may be applicable to SYNGAP1

Focused on products with more historical data; limited label expansion

- **Analysis caveats:**
  - Data availability: analyst estimate look-back and actual sales near peak are often limited (esp. given prolonged 7+ year uptake to peak often seen in rare diseases)
  - Excluded products (e.g., Solris) where initial estimates assumed single indications and product eventually expanded to additional distinct indications
  - Included some indication expansion products when model is still driven by core initial indication + there are SYNGAP1 relevant learnings
  - Also, looked at SMA market where fast followers / multiple products were in sequence in the pipeline to get at potential dynamics in a multiple modality DMT market*

* Excluded: TTR given cardio expansion and Alnylam cannibalizing its own products; Dravet seizure focused market given short sequence of Epidiolex/ Fintepla launches (eventual mkt leader), also approved for LGS

No clear trend; analyst over & underestimates can inform SYNGAP1

- Analysts **both** over and underestimate 1st entry rare disease product revenue; **no consistent trend emerged**
- Misses in both directions are often due to TAM and penetration estimate mismatches
  - Specifically related to initial populations modeled (e.g., more severe, pediatric), lower initial epi estimates
    - Both early Strensiq and Crysvita models did not assign much value to adult mkt; limited overall population literature epi; patient identification improved post-launch
  - Highlights importance of augmenting genetic testing and patient identification pre-launch
- Geographic expansion often conferred upside vs. initial launch forecasts
- Premium pricing is frequently applied in analyst models and does not typically underly forecast misses*

* note: there are specific instances – e.g., weight-based dosing + under-calling adult penetration – that resulted in low initial annual price / patient assumptions (not as relevant to IT ASOs)

Estimate mismatch often due to initial addressable mkt & epi assumptions

Select 1st Entry Products — *WW Annual Sales*

	Product	Disease	Launch Year	Pre-launch Peak Est. (Year Made)		Actual Sales (2025)	Current Peak Est. (Est. Peak Year)	Rationale *Prior vs. actual $ / current estimates*
Under (vs. actual)	**Strensiq** *SC ERT*	Hypophosphatasia (HPP)	2015	$0.5 – 1B (2012-14)	↗	$1.7B	$2B (2027)	**TAM expansion + epi:** early models focused on most severe / pediatric; added adults to later models (note: no label change); early models relied on sparse epi lit. estimates.
Under (vs. current est.)	**Crysvita** *SC mAb*	X-linked hypophosphatemia (XLH)*	2018	$1.2 – 1.8B (2016-2018)	↗	$1.6B	$2 – 3B (2032)	**TAM + geo. expansion:** initial models assumed limited adult ID (vs. reality); LATAM revenue expansion
Under (vs. current est.)	**Vyjuvek** *Topical Gene Tx*	Dystrophic epidermolysis bullosa (DEB)^	2023	$0.75 – 1B (2023)	↗	$0.4B	$1.7 – 2B (2033)	**Penetration + epi + geo. expansion:** better penetration in the diag. patient pool; expanded models into larger prev. estimates; addition of EU sales
Within Range	**Daybue** *Oral SM*	Rett Syndrome	2023	$0.4 – 0.8B (2022-23)	—	$0.4B	$0.5 – 0.9B (2032)	**Pricing vs. tolerability :** pre-launch models assumed $150k / pt / yr; vs. $375k; yet real-world tolerability has hampered uptake; epi largely accurate
Over (vs. current est.)	**Givlaari** *SC siRNA*	Acute hepatic porphyria (AHP)	2019	$0.5 – 1.5B (2017-19)	↘	$0.3B	$0.6B (2033)	**Diagnosis overestimates:** difficult patient ID due to symptom overlap with other diseases, acute attacks; overestimated ALNY gene Tx efforts
Over (vs. current est.)	**Skyclarys** *Oral SM*	Friedreich Ataxia	2023	$1.4 – 2.2B (2022)	↘	$0.5B	$1 – 1.5B (2033)	**Penetration:** earlier estimates overestimated penetration into older / community center patients; poor real-world adherence

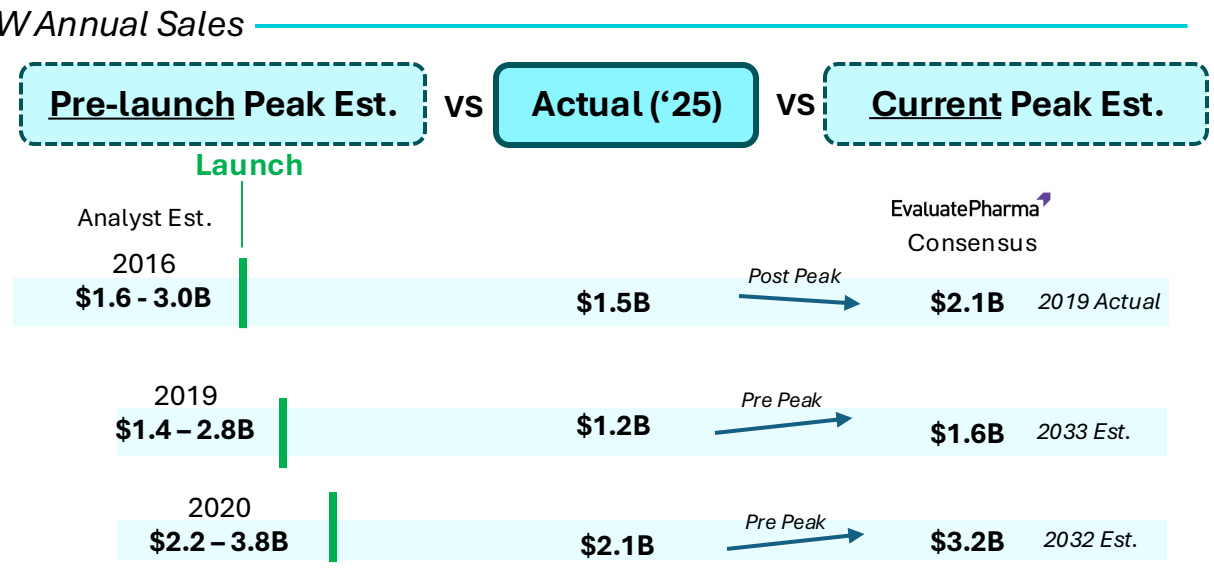
*Crysvita label expanded to Tumor-Induced Osteomalacia in 2020 / expanded within XLH to infants; marginally contributing to sales est.; initial initiation still remains core to $
^ Vyjuvek label expanded to newborns; however, penetration assumptions into initial core age group have increased
Sources: pre-launch : AlphaSense consolidation of analyst estimates (not probability adjusted); actual sales and current consensus forecasts from Evaluate Pharma and select reports

CAMP4

Fast Follower Market

SMA estimates in line with overall mkt size, not product shares

Product	Disease	Launch Year
Spinraza *ITASO*	SMA	2016
Zolgensma *IT Gene Tx*	SMA	2019
Evrysdi *Oral*	SMA	2020



- **Available pre-launch analyst peak sales est. are near launch** – i.e., not a robust pre-launch look-back data set
- **Ranges in estimates reflect multiple products in clinical development simultaneously**; different calls on mkt leader
- Higher analyst pre-launch Spinraza estimates assumed it would continue to take most of the market
- Additional entrants (esp. Evrysdi) had rapid uptake and became SMA market leader

Sources: pre-launch : AlphaSense consolidation of analyst estimates (not probability adjusted); actual sales and current consensus forecasts from Evaluate Pharma

Appendix

Confidential & Proprietary

Strensiq (launched 2015)

Analyst models for **Strensiq** (asfotase alfa) evolved from an "ultra-orphan" niche asset with a $500 million peak sales target into a blockbuster franchise with estimates reaching $1.6 billion before the focus shifted to next-generation follow-ons. This multi-year upward revision was driven primarily by a fundamental reappraisal of the addressable patient population, the impact of weight-based pricing as children aged, and the "stacking" effect of improved survival rates on long-term prevalence. Credit Suisse Research• 2 Sep 14 › Credit Suisse Research• 19 Jan 16 ›

Evolution of Strensiq Peak Sales Estimates

Historical forecasts followed a clear upward trajectory as the drug transitioned from a clinical candidate to a commercial success that repeatedly outperformed launch expectations. Deutsche Bank Research• 3 Nov 16 ›

- **Initial Projections (2012–2014):** Early models in 2012 estimated peak sales at **$625 million**, assuming a 2014 pediatric filing. JPMorgan Research• 16 Apr 12 ›
- **Initial Projections (2012–2014) (cont.):** By late 2014, **Credit Suisse** modeled peak sales of **$500 million** by 2020, based on a 2015 launch. Credit Suisse Research• 2 Sep 14 ›
- **Expansion Phase (2015–2016):** Following Breakthrough Therapy designation and impressive clinical survival data (97% survival at one year), **Bank of America** projected peak sales of **$1.6 billion**, modeling a blockbuster opportunity. Bank of America Global Research• 30 Sep 14 ›
- **Expansion Phase (2015–2016) (cont.): Morgan Stanley** similarly raised its view from $750 million in 2013 to **$1.2 billion** by 2015. Morgan Stanley Research• 25 Oct 13 › Morgan Stanley Research• 23 Apr 15 ›
- **Maturity (2018–2024):** By 2018, **Credit Suisse** had revised its long-term peak sales forecast to **$1.3 billion** by 2028. Credit Suisse Research• 17 Jan 18 ›
- **Maturity (2018–2024) (cont.):** Actual 2024 revenues reached **$1.4 billion**, a 23% year-over-year increase, significantly exceeding early lifecycle projections. TD Cowen• 2 Oct 25 ›

Epidemiology and Diagnosis Rates

Early models relied on sparse literature that estimated the severe form of hypophosphatasia (HPP) at **1 in 100,000** live births. JPMorgan Research• 16 Apr 12 ›

Analysts noted that true prevalence is typically underestimated for ultra-rare diseases because there is no incentive to diagnose until a treatment exists. As **Alexion** educated physicians on "low alkaline phosphatase" levels, diagnosis rates increased, and analysts shifted from incidence-only models to "prevalence stacking," where incidence began to exceed death rates due to the therapy, allowing the treated patient pool to grow cumulatively. Credit Suisse Research• 19 Jan 16 ›

Population Expansion (The Adult Question)

The most significant modeling shift involved the integration of adults, though this remained a point of debate. Credit Suisse Research• 19 Jan 16 ›

- **Label Constraints:** Strensiq is approved for patients with symptom onset before age 18, which technically includes adults who were diagnosed as children. TD Cowen• 2 Oct 25 ›
- **Off-Label Reality:** While roughly **80%** of HPP patients have pediatric-onset disease that continues into adulthood, analysts initially viewed the adult market as a "work in progress" with less clear regulatory paths. JPMorgan Research• 16 Apr 12 ›
- **Conservative Integration: Bank of America** modeled only a **15%** treatment rate for moderate HPP patients, with 65% of those being juvenile-onset and 35% being adult-onset. Bank of America Global Research• 30 Sep 14 ›
- **Conservative Integration (cont.): Credit Suisse** was more cautious, excluding most mild/adult patients from their $1.2 billion peak model but noting that any uptake in this "much more prevalent population" represented pure upside. Credit Suisse Research• 19 Jan 16 ›

Strensiq – CS 2016 historical model build

Restricted to <18 patients given limited information <u>not</u> because of label

CREDIT SUISSE

19 January 2016

How we model Strensiq:

Figure 38: Credit Suisse Modeling Assumptions for Strensiq

	Reasoning	Assumption	Output
Prevalence	Wide prevalence ranges and very limited literature. Alexion guides for 1/100k prevalence based on the Mornet (2007) paper in the U.S.	1/100k	730 Patients under 18 in the U.S. (our checks suggests primary target would be patients under 18 despite broad label) Prevalence growing due to increased survival
Diagosed Patients	Disease is often confused for osteoporosis or simply rickets in the childhood/juvenile form.	65% Diagnosis rate at peak	370 Diagnosed patients (US)
Peak Share	HPP is a severe disease in many patients but there are milder manifestations as well. We aren't sure what the exact breakdown is of mild and severe patients in the population.	80% Share at peak (50-70% ex US) as we are only modeling the prevalence under 18 so we assume these are the severe patients	~600 Patients treated in 2020 (US) due to growing prevalence
Average Price	The price per Mg is $70, but as the drug is dosed Mg/kg, as patients gain weight, the average price will increase.	We assume net price is ~$280k/year in line with guidance but that price grows as average weight increases (and eventually higher discounts to the gross price)	2016 Average price (gross): $480k/year
Peak Sales	**2025: $1.2B Globally**		
US/ Ex-US Split	To be conservative we model price conservatively ex-US. Whereas Soliris sales are ~70% ex-US we model slightly less than this for Strensiq to be conservative.	~60% of sales ex-US	$500M US $700M Ex-US

Source: Company data, Credit Suisse estimates.

CAMP4

Crysvita (launched 2018)

Shifts in Epidemiology and Total Addressable Population

Refinements in the treatable patient population were a primary driver for modeling changes as analysts gained clarity on the number of "lost to follow-up" adults. Expert Call• 4 Aug ›

- **Total Addressable Market (TAM):** Current company data identifies approximately 48,000 patients with XLH in the developed world, comprised of 36,000 adults and 12,000 children. The TIO market adds an additional 2,000 to 4,000 addressable patients globally. 10Q• 5 Aug ›
- **Adult Patient Mix:** Early 2018 models conservatively assumed adults would be harder to identify; however, by 2021, the mix reached roughly 50% adults and 50% pediatric patients, supported by AI-enabled identification and community outreach. CIRC• 6 Aug ›
- **Geographic Expansion:** Growth models were updated to reflect expansion into **Latin America**, which generated $177 million in revenue for **Ultragenyx** in 2025 following successful reimbursement negotiations in Brazil and Mexico. Transcript - Earnings Calls• 5 Aug 25 › Transcript - Earnings Calls• 12 Feb ›

Changes in Penetration and Pricing Assumptions

Analysts adjusted penetration and pricing models based on the drug's superiority over the previous standard of care and real-world weight-based dosing requirements. Credit Suisse Research• 18 Apr 19 ›

- **Penetration Rates:** In 2018, **Credit Suisse** modeled peak pediatric penetration at 80% with a much lower 40% for adults; by 2022, they noted that adult penetration had reached 15% within four years, while pediatric penetration hit 40%. Credit Suisse Research• 29 Jul 22 › Credit Suisse Research• 17 Jan 18 ›
- **Net Realization:** Early 2018 estimates from **Wedbush** assumed a net price of $110,000 per year, but actual commercial metrics showed net pricing exceeding initial "reference" targets ($160,000 for peds and $200,000 for adults) due to higher-than-expected adherence and higher clinical doses. Wedbush• 7 Mar 18 ›
- **Dosing Impact:** By 2026, **DNB Carnegie** estimated that U.S. list price increases and weight-based dosing resulted in annual net revenue of approximately $245,000 per pediatric patient (weighing 20-50kg) after accounting for a 30% gross-to-net discount. DNB Carnegie Sponsored Research• 2 Oct ›

Summary of Historical Modeling Drivers

Driver	Initial Assumption (c. 2018)	Revised Assumption (c. 2022–2026)	Source
Global Peak Sales	~$1.2B – $1.8B	~$2.2B – $3.0B	TD Cowen• 11 Mar 19 ›
Patient Mix	80% Peds / 20% Adult	~50% Peds / 50% Adult	Credit Suisse Research• 3 Aug 21 ›
Net Price (US)	~$110,000 – $160,000	~$200,000 – $245,000	Wedbush• 7 Mar 18 ›
XLH Prevalence	~9,000 US Adults	~36,000 Global Adults	10Q• 5 Aug › Credit Suisse Research• 8 Nov 19 ›
New Indications	Core XLH only	XLH + TIO + Infant XLH	TD Cowen• 11 Mar 19 › Press Release• 21 Jul ›

Vyjuvek (launched 2023)

Research Plan ›

Analysts have steadily revised their peak sales estimates for **Vyjuvek** upward since its initial approval, transitioning from early projections of approximately $750 million to current "Base Case" expectations of $1.4 billion and "Upside" scenarios reaching $1.74 billion. This upward trajectory is driven by a combination of higher-than-expected patient compliance, expansion into a maintenance-dosing model that extends the therapy's "tail," and a clear path for global expansion in Europe and Japan. Jefferies Research• 16 Oct › Transcript - M&A/Special• 19 May 23 › Transcript - Earnings Calls• 4 Aug 25 › Transcript - Earnings Calls• 6 May 25 ›

Penetration and Compliance Assumptions

The most significant change in analyst modeling is the transition from "active treatment" to "maintenance dosing" expectations. RBC Capital Market Research• 14 Aug ›

- **Penetration Rates:** Jefferies' current base case assumes 65% US penetration, while their upside scenario models 75%. Jefferies Research• 16 Oct ›
- **Penetration Rates (cont.):** As of Q2 2026, Krystal had already secured over 730 reimbursement approvals, surpassing its original 60% two-year target. H.C. Wainwright• 3 Aug ›
- **Maintenance Dosing Impact:** Early models assumed continuous weekly dosing; however, real-world data showed patients achieve wound closure faster than expected, leading to "drug holidays." TD Cowen• 20 May ›
- **Maintenance Dosing Impact (cont.):** RBC Capital Markets now models maintenance dosing at roughly 50% of the cost of the first 12–18 months of treatment. RBC Capital Market Research• 14 Aug ›
- **Compliance Stabilization:** Clinical experts suggest that while compliance was near 100% at launch, it has settled into a 60–80% range as patients move to as-needed utilization. Expert Call• 20 Mar › Expert Call• 20 Mar ›

Epidemiology and Population Expansion

Analyst models have evolved from focusing solely on the ~1,200 identified US patients to incorporating a broader prevalence pool and additional indications. H.C. Wainwright• 3 Aug ›

- **Identified vs. Prevalence:** Management and analysts initially modeled a "diagnosed" population of 1,200 patients in the US, but the broader prevalence is estimated at ~3,000. TD Cowen• 20 Feb ›

Givlaari (launched 2019)

Research Plan ›

Alnylam's diagnosis and genetic testing efforts for **Givlaari** (givosiran) have encountered significant structural hurdles, primarily due to the ultra-rare nature of **Acute Hepatic Porphyria (AHP)** and the characteristic symptom overlap with more common conditions like irritable bowel syndrome. While genetic testing initiatives were designed to accelerate patient identification, analysts note that the revenue growth trajectory has moderated, suggesting the company may have reached the upper limits of the easily identifiable patient population faster than initially modeled. Expert Call• 9 Aug 24 › Expert Call• 29 Jul 25 ›

Diagnosis and Patient Identification Challenges

The primary difficulty in identifying AHP patients stems from the disease's presentation, which often involves recurring, excruciating abdominal pain that physicians frequently mistake for more prevalent gastrointestinal issues. Transcript - Analyst/Investor• 6 May 20 ›

- **Clinical Awareness Gap:** Many physicians have never encountered a case of AHP, having only learned about it briefly in medical school, which leads to a prolonged "patient journey" before a definitive diagnosis is considered. Expert Call• 12 Mar 25 ›
- **Symptom Overlap:** Alnylam's commercial strategy focused on educating physicians to consider AHP when a patient with irritable bowel-like symptoms failed to improve despite repeated changes in disease approach. Expert Call• 9 Aug 24 ›
- **Therapeutic Inertia:** Even after diagnosis, commercial uptake can be limited by a reluctance to switch patients from existing chronic pain management regimens to a new RNAi therapeutic. Expert Call• 12 Mar 25 ›

Skyclarys (launched 2023)

Analyst estimates for **Skyclarys** (omaveloxolone) peak sales have transitioned from early "blue sky" projections of over $2 billion to a more tempered consensus of approximately $1.2 billion as the commercial reality of the launch has set in. While the drug remains a high-growth asset for **Biogen**, several factors—including a shift in the U.S. patient demographic, real-world discontinuation rates due to liver enzyme elevations, and complex international reimbursement dynamics—have led analysts to refine their long-term models. TD Cowen• 29 Jan › Expert Call• 19 Oct › TD Cowen• 29 Jul ›

Evolution of Market Penetration and Patient ID Challenges

The initial rapid uptake of Skyclarys in the U.S. was driven by a "catch-up" bolus of patients at academic centers of excellence who were ready for treatment immediately upon approval. Analysts have identified several hurdles to sustaining this growth: Transcript - Conference• 10 Mar 25 ›

- **Saturation of Centers of Excellence:** Management indicates that the launch has moved into a "searching for patients" phase, as approximately 90% of the remaining U.S. opportunity now lies within community clinics rather than specialized centers. TD Cowen• 29 Jan › Transcript - Earnings Calls• 30 Oct 25 ›
- **Milder Community Phenotype:** These remaining community-based patients are often older (over age 45) and exhibit a milder form of Friedreich's ataxia (FA) than previously appreciated, which may lead to a lower sense of urgency for treatment among some clinicians. TD Cowen• 29 Jul ›
- **Lower Candidate Proportion:** Physician surveys in late 2024 indicated that while market share grew, the estimated proportion of total FA patients considered viable candidates for Skyclarys remained flat at roughly 42%. Wedbush• 22 Sep 24 ›
- **Shift in Prescriber Base:** To reach these community patients, **Biogen** has pivoted its investment toward community neurologists and primary care physicians (PCPs), who accounted for roughly 70% of new U.S. starts in the second quarter of 2025. Earnings Presentation• 31 Jul 25 ›

Epidemiology and Addressable Market Updates

Refinements in epidemiological data have shifted the focus toward different age groups and geographic regions:

- **Prevalence of Older Survivors:** Historically, the life expectancy for FA patients was cited as 38 to 39 years; however, data now shows a significant population of survivors aged 45 and older living with milder disease, suggesting a slightly larger but harder-to-reach prevalence pool than initially modeled. Jefferies Research• 9 Oct ›

Real-World Persistence and Safety Headwinds

Analyst revisions have also begun to incorporate real-world data regarding patient adherence and side effects:

- **Liver Enzyme Elevations:** Expert neurologists report that a significant portion of patients—in some cases up to 75% in specific clinical practices—have had to discontinue Skyclarys due to elevated liver enzymes (ALT/AST). Expert Call• 19 Oct ›
- **Dose Titration and Moderation:** Instead of complete discontinuation, some physicians manage side effects by reducing the dose (e.g., from three capsules daily to one), which impacts the overall volume of drug sold. Transcript - Conference• 10 Mar 25 ›
- **Adherence Rates:** Survey data suggests that only about 48% of patients remain on therapy after six months, creating a significant headwind to long-term peak sales targets. Wedbush• 22 Sep 24 ›

EvaluatePharma actual and consensus sales est.

EvaluatePharma

All Financial Data in US $ (mln)

			Annual Sales (Indication) - WW - Sales																					
Rank	Product	Company	2011	2012	2013	2014	2015	2016	2017	2018	2019	2020	2021	2022	2023	2024	2025	2026	2027	2028	2029	2030	2031	2032	2033
1	Evrysdi	Roche	-	-	-	-	-	-	-	-	-	59	638	1,089	1,477	1,747	2,011	2,423	2,570	2,707	2,832	2,957	3,089	3,191	3,113
2	Zolgensma	Novartis	-	-	-	-	-	-	-	-	361	920	1,351	1,370	1,214	1,214	1,232	1,240	1,286	1,303	1,334	1,411	1,498	1,567	1,592
3	Isembyld	Scholar Rock	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	8	163	403	684	891	1,088	1,272	1,405
4	Spinraza	Biogen	-	-	-	-	-	5	884	1,724	2,097	2,052	1,905	1,794	1,741	1,573	1,547	1,564	1,466	1,387	1,309	1,252	1,199	1,149	1,101
5	Itvisma	Novartis	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	215	456	796	1,059	890	749	653	540
6	Evrysdi	Chugai Pharmaceutical	-	-	-	-	-	-	-	-	-	-	21	90	103	105	108	101	100	101	102	101	100	93	84
7	Salanersen	Biogen	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	17	33	50	67
8	Adimanebart	argenx	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	1	1	2	3	3	4	5
Total			-	-	-	-	-	5	884	1,724	2,458	3,031	3,915	4,342	4,535	4,640	4,898	5,551	6,041	6,699	7,321	7,521	7,760	7,978	7,908

Spinraza Analyst Estimates

Historical Analyst Peak Sales Estimates (Pre-Launch & Early Launch)

AlphaSense

Product	Estimate Date	Analyst Source	Peak Sales Estimate	Key Context	Source
Spinraza	Jan 2016	Credit Suisse	$3.00B	Pre-launch (approx. 11 months prior to FDA approval).	Credit Suisse Research· 19 Jan 16 ›
	Dec 2016	JPMorgan	~$3.00B	Published 27 Dec 2016, immediately following early FDA approval.	JPMorgan Research· 27 Dec 16 ›
	Feb 2017	Credit Suisse	$2.50B	Base case estimate shortly after launch; noted consensus was higher at $1.8B for 2020.	Credit Suisse Research· 3 Feb 17 ›
	Sep 2017	RBC Capital	$1.60B	Base case reflecting concerns over high-cost maintenance and limited patient starts.	RBC Capital Market Research· 14 Sep 17 ›
	Oct 2017	BofA	$1.90B	Maintained projection following positive interim data from EMBRACE trial.	Bank of America Global Research· 5 Oct 17 ›
	Nov 2017	JPMorgan	~$3.00B	Reiterated healthy peak sales expectations despite strong initial launch ramp.	JPMorgan Research· 9 Nov 17 ›
	Dec 2017	Credit Suisse	$3.20B	Revised peak for 2025 following strong first year of launch.	Credit Suisse Research· 20 Dec 17 ›

Zolgensma Analyst Estimates

Historical Analyst Peak Sales Estimates (Pre-Launch & Early Launch)

AlphaSense

Product	Estimate Date	Analyst Source	Peak Sales Estimate	Key Context	Source
			Outlier		
Zolgensma	Apr 2018	BofA	$5.00B	Un-risk-adjusted peak following Novartis's acquisition of AveXis.	
	Apr 2019	Jefferies	$2.60B	Estimated WW peak by 2024E; noted it was well above consensus of $1.9B.	Jefferies Research· 24 Apr 19 ›
	Apr 2019	Morgan Stanley	$1.40B	Forecasted 15% lower than consensus due to upcoming competition.	Morgan Stanley Research· 10 Apr 19 ›
	Aug 2019	Jefferies	$2.80B	Revised upward following US approval; assumed 80% Type 1 penetration.	Jefferies Research· 30 Aug 19 ›
	Sep 2019	BofA	$2.50B	Combined $0.9B (IV) and $1.6B (IT) formulation peak for 2024E.	Bank of America Global Research· 13 Sep 19 ›
	Mar 2020	BofA	$2.10B	Refined combined peak forecast including pending intrathecal (IT) version.	Bank of America Global Research· 24 Mar 20 ›
	Jan 2021	Deutsche Bank	$1.70B	Estimated peak for 2025; noted it was below the $2B management target.	Deutsche Bank Research· 14 Jan 21 ›

Evrysdi Analyst Estimates

Historical Analyst Peak Sales Estimates (Pre-Launch & Early Launch)

AlphaSense

Product	Estimate Date	Analyst Source	Peak Sales Estimate	Key Context	Source
Evrysdi	Jan 2020	BofA	$2.80B	Forecast for 2027E, assuming 35% penetration into prevalent population.	Bank of America Global Research• 21 Jan 20 ›
	Mar 2020	BofA	$2.90B	Revised peak for 2027E prior to US approval (August 2020).	Bank of America Global Research• 13 Sep 19 ›
	Jul 2020	Bernstein	~$2.17B	Stated as CHF 2B peak sales at 90% probability of approval.	Bernstein Research• 22 Jul 20 ›
	Jul 2020	Credit Suisse	~$3.80B	Stated as $3.5B peak for 2034E prior to monetization of royalties.	Credit Suisse Research• 20 Jul 20 ›
	Aug 2020	Credit Suisse	~$2.80B	Revised net price assumption to $280K/year post-launch approval.	Credit Suisse Research• 10 Aug 20 ›
	Feb 2021	TD Cowen	$3.20B	Base case peak revenue assumption.	